



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : ايه علاء عبدالرشيد محمد
المحافظة : القليوبية الحي / المدينة : القناطر الخيرية العنوان :
رقم البطاقة : 29402151400728 رقم التليفون : 01127879761 تاريخ تسجيل الإستمارة : 24-09-2026
العمر (تاريخ الميلاد) : 15-02-1994 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي (.....) التوقيع: (.....)
المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الأورام

خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر والحوض
بالصبغة [11127]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / أ.د. / أ.د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ايه علاء عبدالرشيد محمد National ID: 29402151400728	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



ابيه
علام عبدالرشيد محمد
ابوالغيط
مركز القناطر الخيرية - القليوبية

١٩٩٤/٠١
٢٩٤٠٢١٥١٤٠٠٧٢٨

K00209568



[[DocType: ID - Back]]

BQHEYA

Radiology Department

ب.هية
قسم الأشعة الإشعاعية

PATIENT NAME: aya ALAA ABDELRAHED MOHAMMED

GENDER: Female

DATE : 03-Sep-2026 12:49 PM

PATIENT ID: 10126284858

AGE: 32 y

ORDER: 4020431,4020689

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Screening.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).
- Right breast UIQ two deeply seated spiculated masses by U/S revealed two ill defined Hetrogenous masses at 2 o'clock surrounded with echogenic rind are seen measuring 15x11.5 mm & 13x8 mm as well as diffuse thickened skin and internal vascularity on colour doppler
- Multiple variable sized pathological lymph nodes are seen at level (I-II-III) the largest at the axilla measuring 46x26 mm
- Multiple ipsilateral prominent upper deep cervical lymph nodes the largest measures 22x6 mm
- Bilateral scattered variable sized anechoic cysts and no left solid masses at time of scan
- Normal skin thickness and contour of left breast .
- Normal nipple areola complexes.
- Left axilla :Nonspecific axillary lymph nodes.

OPINION:

- Right breast UIQ multi focal malignant process as described for biobsy and multiple ipsilateral pathological lymph nodes at (level I-II-III) (BIRADS 5).
- Right Prominnat upper deep cervical lymph nodes
- Bilateral fibro cystic changes (BIRADS 2)

PATIENT: aya ALAA ABDELRAHED MOHAMMED, 10126284858, Female, 32 y

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مركز تهيئة الثدي
4 شارع علوية أول شارع الهرم بجوار داري، الجزيرة
مستشفى تهيئة الثدي زايد
4. الحي الأول - الشيخ زايد أمام كلية الهندسة



RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Mahetab Afifi Mohamed, MSc, FEBR

03-Sep-2026 02:47 PM

PATIENT: aya ALAA ABDELRAHED MOHAMMED, 10126284858, Female, 32 y

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مركز بحية الهرم
4 شارع طلوية أول شارع الهرم بجوار داري، الجزيرة
مستشفى بحية الشيخ زايد
4. الحيا الأولى - الشيخ زايد أمام كلية الهندسة



BQHEYA

Radiology Department

ب.ه.ي.ة
قسم الأشعة التشخيصية

PATIENT NAME: aya ALAA ABDELRAHED MOHAMMED

PATIENT ID: 10126284858

GENDER: Female

AGE: 32 y

DATE : 06-Sep-2026 10:57 AM

ORDER: 4020819,4020820

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC LIVER STUDY

FINDINGS:

1-CHEST

- Right breast UIQ two irregular enhancing nodules the largest measuring 13 x 10 mm.
- Multiple right axillary level I, II & III pathological lymph nodes the largest is at level I measuring 53 x 23 mm.
- Multiple right internal mammary pathologically enlarged lymph nodes the largest measuring 17 x 10 mm.
- Bilateral supraclavicular subcentimetric lymph nodes largest at left measuring 7 x 4 mm.
- Few bilateral millimetric pulmonary nodules largest at right upper lobe anterior segment measuring 3 mm.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- Few small prominent anterior mediastinal and retrosternal lymph nodes the largest measuring 11 x 9 mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation..
- Patent homogeneously enhancing portal vein and is main branches as well as its main tributaries.

PATIENT: aya ALAA ABDELRAHED MOHAMMED, 10126284858, Female, 32 y

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مركز تهية الهدى
4 شارع علوية أول شارع الهرم بجوار داري ، الجيزة
مستشفى تهية الشيخ زايد
4: الحى الأول - الشيخ زايد أمام كلية الهندسة



- Normal appearance of the pancreas, spleen, kidneys, adrenals, aorta, I.V.C and the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- Few small prominent para aortic lymph nodes.
- No abdominal or pelvic masses or collections.
- **The revised bone window revealed:**
 1. L4 & D12 lytic osseous lesions with cortical breaching and extra osseous soft tissue sheets at L4.
 2. No CT evidence of vertebral collapse.
 3. Left acetabular roof sclerotic focus likely bone island.

Opinion:

- *Right breast neoplastic lesions with ipsilateral pathological axillary lymph nodes.*
- *Multiple right internal mammary pathologically enlarged lymph nodes.*
- *Bilateral supraclavicular subcentimetric lymph nodes.*
- *Few small prominent anterior mediastinal and retrosternal lymph nodes for follow up.*
- *Few bilateral millimetric pulmonary nodules likely nonspecific for follow up.*
- *L4 & D12 lytic osseous lesions likely metastatic.*

Reported By,

Dr. Ali Saber, MSc

Dr. Hekmat Samir, MD

07-Sep-2026 11:59 AM

PATIENT: aya ALAA ABDELRAHED MOHAMMED, 10126284858, Female, 32 y

Report version 2, 07-Sep-2026 11:59 AM

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مركز طبية الهدى
4 شارع علوية أبو عمار الهدى بجوار داري، الحجرة
مستشفى طبية الشيخ زايد
4، الحي الأول - الشيخ زايد أمام كلية الهندسة



BaHEYA

Pathology Department
 قسم الباثولوجي

ب.هية

MRN: 10126284858 D.O.B: 15/02/1994
 Patient Name: aya ALAA ABDELRAshed MOHAMMED
 IC No.: 29402151400728 Sex: Female
 Location: External Clinic

SURGICAL PATHOLOGY
Biopsy Small - (1 Site)

Specimen Id 2609060184 Collection Date 06/09/2026 02:15 PM
 Accession No SUR-26-3253 Received Date 06/09/2026 02:15 PM
 Sample Type Tissue Doctor DALIA MAHMOUD MOHAMED
 MAHMOUD

Clinical Information
 Rt breast 2 o'clock highly suspicious lesion (BIRADS 5).

Gross Picture
 Fragmented tissue cores 0.3-1 cm

Microscopic Picture
 A case of invasive duct carcinoma, grade 3. No in-situ component nor lymphovascular emboli were detected. Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnostic Summary

Tumour type:
 Invasive duct carcinoma

Laterality
 Right

Specimen type:
 US-guided biopsy

Location
 2 o'clock

Histological grade

Mitosis score (1-3):
 2

Tubular formation score (1-3):
 3

Nuclear grade (1-3):
 3

Histological grade (1-3):

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مركز هيئة اليوم
 4 شارع علوية اول شارع اليوم بجوار دارى, الجيزة
 مستشفى هيئة الشيخ زايد
 ٤. الحي الأول - الشيخ زايد أمام كلية الهندسة



3

Peritumoural lymphovascular invasion

Absent

Diagnosis

Rt breast (2 o'clock), US-guided biopsy :

Invasive duct carcinoma, grade 3.

Comments

ER, PR and HER2 immunostaining is ordered.

Biopsy reporting category

B5b= Invasive cancer

Signature

Dr. Al Shimaa Zakaria, MD

Co-signed by:

Heba A Abdelbaky (MD)

Reported and electronically endorsed by

heba ahmed Abdel Baqi Abdallah

Pathology lab director:

Dr. Hazem Abdelazim, MD, IFCAP

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16/09/2026 03:50 PM

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مركز تهيئة الهرم
4 شارع علوية أول شارع الهرم بجوار دولي، الجزيرةمستشفى طبية الشيخ زايد
4. الحي الأول - الشيخ زايد أمام كلية الهندسة

Baheyra

Pathology Department
قسم الباثولوجي

ب.ع.ب.ب.

MRN: 10126284858
Patient Name: aya ALAA ABDELRAshed MOHAMMED
IC No.: 29402151400728
Location: External Clinic

D.O.B: 15/02/1994
Sex: Female

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id 2609080017
Accession No IMM-26-3860
Sample Type Paraffin Block

Collection Date 08/09/2026 08:48 AM
Received Date 08/09/2026 08:48 AM
Doctor Mohamed Mostafa Mohamed Mostafa

Immunohistochemical Report

pathology number
3253-26

Clinical Data
Rt. breast cancer.

Gross Picture :
Referred block submitted for ER/PR immunostaining.

System Used :
Ventana Benchmark ULTRA autostainer.

Antibody:
Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :
According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:
- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

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مركز طبقة الهرم
4 شارع علوية أول شارع الهرم بجوار دارى, الحجرة
مستشفى طبقة الشيخ زايد
٤. الحى الأول - الشيخ زايد أمام كلية الهندسة

- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:
Right

Diagnosis :

ER: Negative [0/8].

PR: Negative [0/8].

Signature

Heba A Abdelbaky (MD)

Reported and electronically endorsed by

heba ahmed Abdel Baqi Abdallah

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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Baheyra

Pathology Department
قسم الباثولوجي

ب.إ.ح.ب.

MRN: 10126284858
Patient Name: aya ALAA ABDELRAHED MOHAMMED
IC No.: 29402151400728
Location: External Clinic

D.O.B: 15/02/1994
Sex: Female

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 2609080018
Accession No IMM-26-3861
Sample Type Paraffin Block

Collection Date 08/09/2026 08:48 AM
Received Date 08/09/2026 08:48 AM
Doctor Mohamed Mostafa Mohamed Mostafa

Immunohistochemical Report

pathology number
3253-26

Clinical Data
Rt. breast cancer.

Gross Picture :
Referred block submitted for HER 2 immunostaining.

System Used :
Ventana Benchmark ULTRA autostainer.

Antibody:
Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :
Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining that is observed in $>10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in $>10\%$ of tumor cells.

(Laterality) breast Cancer:
Right
Laterality :
Right
Her2 Result
Positive [score 3+]

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مركز رعاية الهرم
4 شارع طلوعة أول شارع الهرم بجوار دارى , الجزيرة
مستشفى مدينة الشيخ زايد
٤٠ الحى الأول - الشيخ زايد أمام كلية الهندسة



Signature

Heba A Abdelbaky (MD)

Reported and electronically endorsed by
heba ahmed Abdel Baqi Abdallah

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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مركز بنية الهرم
4 شارع طلوية أول شارع الهرم بحوار داري , الجزيرة
مستشفى بنية الشيخ زايد
٤٠ الحى الأول - الشيخ زايد أمام كلية الهندسة



جمهورية مصر العربية
بطاقة تحقيق الشخصية



أيه
علام عبد الرشيد محمد
أبو الغيط
مركز القناطر الخيرية - القليوبية

١٩٩٤/٠١
٢٩٤٠٢١٥١٤٠٠٧٢٨

K00209568



PATIENT NAME: aya ALAA ABDELRAHED MOHAMMED

GENDER: Female

DATE : 03-Sep-2026 12:49 PM

PATIENT ID: 10126284858

AGE: 32 y

ORDER: 4020431,4020689

**FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND****INDICATION:**

- Screening.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).
- Right breast UIQ two deeply seated spiculated masses by U/S revealed two ill defined Hetrogenous masses at 2 o'clock surrounded with echogenic rind are seen measuring 15x11.5 mm & 13x8 mm as well as diffuse thickened skin and internal vascularity on colour doppler
- Multiple variable sized pathological lymph nodes are seen at level (I-II-III) the largest at the axilla measuring 46x26 mm
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- Bilateral scattered variable sized anechoic cysts and no left solid masses at time of scan
- Normal skin thickness and contour of left breast .
- Normal nipple areola complexes.
- Left axilla :Nonspecific axillary lymph nodes.

OPINION:

- Right breast UIQ multi focal malignant process as described for biobsy and multiple ipsilateral pathological lymph nodes at (level I-II-III) (BIRADS 5).
- Right Prominnant upper deep cervical lymph nodes
- Bilateral fibro cystic changes (BIRADS 2)

PATIENT: aya ALAA ABDELRAHED MOHAMMED, 10126284858, Female, 32 y

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مركز طبية الهديا
4 شارع علوية أول شارع الهرم بجوار داري، الجزيرةمستشفى طبية الشيخ زايد
4 الحي الأول - الشيخ زايد أمام كلية الهندسة

RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Mahetab Afifi Mohamed, MSc, FEBR

03-Sep-2026 02:47 PM

PATIENT: aya ALAA ABDELRAHED MOHAMMED, 10126284858, Female, 32 y

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مركز بيهة الهرم
4 شارع طلوية أول شارع الهرم بجوار داري، الجزيرة

مستشفى بيهة الشيخ زايد
4. الحيا الأول - الشيخ زايد أمام كلية الهندسة



PATIENT NAME: aya ALAA ABDELRAHED MOHAMMED

PATIENT ID: 10126284858

GENDER: Female

AGE: 32 y

DATE : 06-Sep-2026 10:57 AM

ORDER: 4020819,4020820

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS WITH TRIPHASIC LIVER STUDY****FINDINGS:****1-CHEST**

Right breast UIQ two irregular enhancing nodules the largest measuring 13 x 10 mm.

- Multiple right axillary level I, II & III pathological lymph nodes the largest is at level I measuring 53 x 23 mm.
- Multiple right internal mammary pathologically enlarged lymph nodes the largest measuring 17 x 10 mm.
- Bilateral supraclavicular subcentimetric lymph nodes largest at left measuring 7 x 4 mm.
- Few bilateral millimetric pulmonary nodules largest at right upper lobe anterior segment measuring 3 mm.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- Few small prominent anterior mediastinal and retrosternal lymph nodes the largest measuring 11 x 9 mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation..
- Patent homogeneously enhancing portal vein and is main branches as well as its main tributaries.

PATIENT: aya ALAA ABDELRAHED MOHAMMED, 10126284858, Female, 32 y

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مركز تشخيص الأشعة
4 شارع علوية أول شارع النور بحول داري، الجيزة
مستشفى تشخيص الأشعة
4 شارع الأول - الشيخ زايد أمام كلية الهندسة



- Normal appearance of the pancreas, spleen, kidneys, adrenals, aorta, I.V.C and the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- Few small prominent para aortic lymph nodes.
- No abdominal or pelvic masses or collections.
- **The revised bone window revealed:**
 1. L4 & D12 lytic osseous lesions with cortical breaching and extra osseous soft tissue sheets at L4.
 2. No CT evidence of vertebral collapse.
 3. Left acetabular roof sclerotic focus likely bone island.

Opinion:

- *Right breast neoplastic lesions with ipsilateral pathological axillary lymph nodes.*
- *Multiple right internal mammary pathologically enlarged lymph nodes.*
- *Bilateral supraclavicular subcentimetric lymph nodes.*
- *Few small prominent anterior mediastinal and retrosternal lymph nodes for follow up.*
- *Few bilateral millimetric pulmonary nodules likely nonspecific for follow up.*
- *L4 & D12 lytic osseous lesions likely metastatic.*

Reported By,

Dr. Ali Saber, MSc

Dr. Hekmat Samir, MD

07-Sep-2026 11:59 AM

PATIENT: aya ALAA ABDELRAHED MOHAMMED, 10126284858, Female, 32 y

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مركز طبية الصدر
4 شارع علوية أبو عمار الشرق بحوار داري، الجيزة
مستشفى طبية الشيخ زايد
4. الحي الأول - الشيخ زايد أمام كلية الهندسة



MRN: 10126284858 D.O.B: 15/02/1994
Patient Name: aya ALAA ABDELRAshed MOHAMMED
IC No.: 29402151400728 Sex: Female
Location: External Clinic

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id 2609060184
Accession No SUR-26-3253
Sample Type Tissue

Collection Date 06/09/2026 02:15 PM
Received Date 06/09/2026 02:15 PM
Doctor DALIA MAHMOUD MOHAMED
MAHMOUD

Clinical Information
Rt breast 2 o'clock highly suspicious lesion (BIRADS 5).

Gross Picture
Fragmented tissue cores 0.3-1 cm

Microscopic Picture
A case of invasive duct carcinoma, grade 3. No in-situ component nor lymphovascular emboli were detected. Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnostic Summary

Tumour type:
Invasive duct carcinoma

Laterality
Right

Specimen type:
US-guided biopsy

Location
2 o'clock

Histological grade

Mitosis score (1-3):
2

Tubular formation score (1-3):
3

Nuclear grade (1-3):
3

Histological grade (1-3):

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مركز هيئة الباثولوجي
4 شارع علوية أول شارع الهرم بجوار دارى, الجيزة
مستشفى هيئة الشيخ زايد
4. الحى الأول - الشيخ زايد أمام كلية الهندسة

3

Peritumoural lymphovascular invasion

Absent

Diagnosis

Rt breast (2 o'clock), US-guided biopsy :

Invasive duct carcinoma, grade 3.

Comments

ER, PR and HER2 immunostaining is ordered.

Biopsy reporting category

B5b= Invasive cancer

Signature

Dr. Al Shimaa Zakaria, MD

Co-signed by:

Heba A Abdelbaky (MD)

Reported and electronically endorsed by

heba ahmed Abdel Baqi Abdallah

Pathology lab director:

Dr. Hazem Abdelazim, MD, IFCAP

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مركز نهية الهرم
4 شارع علوية أول شارع الهرم بجوار دولي, الجزيرة

مستشفى نهية الشيخ زايد
4. الحي الأول - الشيخ زايد أمام كلية الهندسة

MRN: 10126284858
Patient Name: aya ALAA ABDELRAshed MOHAMMED
IC No.: 29402151400728
Location: External Clinic

D.O.B: 15/02/1994
Sex: Female

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id 2609080017
Accession No IMM-26-3860
Sample Type Paraffin Block

Collection Date 08/09/2026 08:48 AM
Received Date 08/09/2026 08:48 AM
Doctor Mohamed Mostafa Mohamed Mostafa

Immunohistochemical Report

pathology number
3253-26

Clinical Data
Rt. breast cancer.

Gross Picture :
Referred block submitted for ER/PR immunostaining.

System Used :
Ventana Benchmark ULTRA autostainer.

Antibody:
Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :
According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:
- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

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مركز رعاية الهرم
4 شارع علوية أول شارع الهرم بحول دادي, الحجرة
مستشفى بريدة الشيخ زايد
٤. الحي الأول - الشيخ زايد أمام كلية الهندسة

- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:
Right

Diagnosis :

ER: Negative [0/8].

PR: Negative [0/8].

Signature

Heba A Abdelbaky (MD)

Reported and electronically endorsed by

heba ahmed Abdel Baqi Abdallah

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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MRN: 10126284858
Patient Name: aya ALAA ABDELRAHED MOHAMMED
IC No.: 29402151400728
Location: External Clinic

D.O.B: 15/02/1994
Sex: Female

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 2609080018
Accession No IMM-26-3861
Sample Type Paraffin Block

Collection Date 08/09/2026 08:48 AM
Received Date 08/09/2026 08:48 AM
Doctor Mohamed Mostafa Mohamed Mostafa

Immunohistochemical Report

pathology number
3253-26

Clinical Data
Rt. breast cancer.

Gross Picture :
Referred block submitted for HER 2 immunostaining.

System Used :
Ventana Benchmark ULTRA autostainer.

Antibody:
Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :
Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining that is observed in $>10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in $>10\%$ of tumor cells.

(Laterality) breast Cancer:
Right

Laterality :
Right

Her2 Result
Positive [score 3+]

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مركز رعاية الهرم
4 شارع طلوع أول شارع الهرم بجوار داري، الجزيرة
مستشفى رعاية الشيخ زايد
٤٠ الحي الأول - الشيخ زايد أمام كلية الهندسة

Signature

Heba A Abdelbaky (MD)

Reported and electronically endorsed by

heba ahmed Abdel Baqi Abdallah

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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مركز بنية الهرم
4 شارع طلوية أول شارع الهرم بحوار داري , الجزيرة
مستشفى بنية الشيخ زايد
٤٠ الحى الأول - الشيخ زايد أمام كلية الهندسة

جمهورية مصر العربية
بطاقة تحقيق الشخصية



أيه

علامه عبدالرشيد محمد

ابوالغيط

مركز القناطر الخيرية - القليوبية

١٩٩٤/٠١



٢٩٤٠٢١٥١٤٠٠٧٢٨

K00209568



PATIENT NAME: aya ALAA ABDELRAHED MOHAMMED

GENDER: Female

DATE : 03-Sep-2026 12:49 PM

PATIENT ID: 10126284858

AGE: 32 y

ORDER: 4020431,4020689

**FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND****INDICATION:**

- Screening.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).
- Right breast UIQ two deeply seated spiculated masses by U/S revealed two ill defined Hetrogenous masses at 2 o'clock surrounded with echogenic rind are seen measuring 15x11.5 mm & 13x8 mm as well as diffuse thickened skin and internal vascularity on colour doppler
- Multiple variable sized pathological lymph nodes are seen at level (I-II-III) the largest at the axilla measuring 46x26 mm
- Multiple ipsilateral prominent upper deep cervical lymph nodes the largest measures 22x6 mm
- Bilateral scattered variable sized anechoic cysts and no left solid masses at time of scan
- Normal skin thickness and contour of left breast .
- Normal nipple areola complexes.
- Left axilla :Nonspecific axillary lymph nodes.

OPINION:

- Right breast UIQ multi focal malignant process as described for biobsy and multiple ipsilateral pathological lymph nodes at (level I-II-III) (BIRADS 5).
- Right Prominnant upper deep cervical lymph nodes
- Bilateral fibro cystic changes (BIRADS 2)

PATIENT: aya ALAA ABDELRAHED MOHAMMED, 10126284858, Female, 32 y

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مركز طبية الهدي
4 شارع علوية أول شارع الهرم بحول داري، الجزيرة
مستشفى طبية الشيخ زايد
4. الحي الأول - الشيخ زايد أمام كلية الهندسة



RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Mahetab Afifi Mohamed, MSc, FEBR

03-Sep-2026 02:47 PM

PATIENT: aya ALAA ABDELRAHED MOHAMMED, 10126284858, Female, 32 y

Report version 1, 03-Sep-2026 02:47 PM

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مركز بيهة الهرم
4 شارع طلوية أول شارع الهرم بجوار داري، الجزيرة

مستشفى بيهة الشيخ زايد
4. الحيا الأول - الشيخ زايد أمام كلية الهندسة



PATIENT NAME: aya ALAA ABDELRAHED MOHAMMED

PATIENT ID: 10126284858

GENDER: Female

AGE: 32 y

DATE : 06-Sep-2026 10:57 AM

ORDER: 4020819,4020820

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS WITH TRIPHASIC LIVER STUDY****FINDINGS:****1-CHEST**

Right breast UIQ two irregular enhancing nodules the largest measuring 13 x 10 mm.

- Multiple right axillary level I, II & III pathological lymph nodes the largest is at level I measuring 53 x 23 mm.
- Multiple right internal mammary pathologically enlarged lymph nodes the largest measuring 17 x 10 mm.
- Bilateral supraclavicular subcentimetric lymph nodes largest at left measuring 7 x 4 mm.
- Few bilateral millimetric pulmonary nodules largest at right upper lobe anterior segment measuring 3 mm.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- Few small prominent anterior mediastinal and retrosternal lymph nodes the largest measuring 11 x 9 mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation..
- Patent homogeneously enhancing portal vein and is main branches as well as its main tributaries.

PATIENT: aya ALAA ABDELRAHED MOHAMMED, 10126284858, Female, 32 y

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مركز تشيعة الهيد
4 شارع علوية اول شارع الهوم بجوار داري ، الجيزة
مستشفى تشيعة الشيخ زايد
4: الحى الأول - الشيخ زايد امام كلية الهندسة



- Normal appearance of the pancreas, spleen, kidneys, adrenals, aorta, I.V.C and the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- Few small prominent para aortic lymph nodes.
- No abdominal or pelvic masses or collections.
- **The revised bone window revealed:**
 1. L4 & D12 lytic osseous lesions with cortical breaching and extra osseous soft tissue sheets at L4.
 2. No CT evidence of vertebral collapse.
 3. Left acetabular roof sclerotic focus likely bone island.

Opinion:

- *Right breast neoplastic lesions with ipsilateral pathological axillary lymph nodes.*
- *Multiple right internal mammary pathologically enlarged lymph nodes.*
- *Bilateral supraclavicular subcentimetric lymph nodes.*
- *Few small prominent anterior mediastinal and retrosternal lymph nodes for follow up.*
- *Few bilateral millimetric pulmonary nodules likely nonspecific for follow up.*
- *L4 & D12 lytic osseous lesions likely metastatic.*

Reported By,

Dr. Ali Saber, MSc

Dr. Hekmat Samir, MD

07-Sep-2026 11:59 AM

PATIENT: aya ALAA ABDELRAHED MOHAMMED, 10126284858, Female, 32 y

Report version 2, 07-Sep-2026 11:59 AM

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مركز طبية الدم
4 شارع علوية أبو عمار الدم بحوار داري، الجيزة
مستشفى طبية الشيخ زايد
4. الحي الأول - الشيخ زايد أمام كلية الهندسة



MRN: 10126284858 D.O.B: 15/02/1994
Patient Name: aya ALAA ABDELRAshed MOHAMMED
IC No.: 29402151400728 Sex: Female
Location: External Clinic

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id 2609060184
Accession No SUR-26-3253
Sample Type Tissue

Collection Date 06/09/2026 02:15 PM
Received Date 06/09/2026 02:15 PM
Doctor DALIA MAHMOUD MOHAMED
MAHMOUD

Clinical Information
Rt breast 2 o'clock highly suspicious lesion (BIRADS 5).

Gross Picture
Fragmented tissue cores 0.3-1 cm

Microscopic Picture
A case of invasive duct carcinoma, grade 3. No in-situ component nor lymphovascular emboli were detected. Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnostic Summary

Tumour type:
Invasive duct carcinoma

Laterality
Right

Specimen type:
US-guided biopsy

Location
2 o'clock

Histological grade

Mitosis score (1-3):
2

Tubular formation score (1-3):
3

Nuclear grade (1-3):
3

Histological grade (1-3):

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مستشفى هيئة الشيخ زايد
4. الحي الأول - الشيخ زايد أمام كلية الهندسة

3

Peritumoural lymphovascular invasion

Absent

Diagnosis

Rt breast (2 o'clock), US-guided biopsy :

Invasive duct carcinoma, grade 3.

Comments

ER, PR and HER2 immunostaining is ordered.

Biopsy reporting category

B5b= Invasive cancer

Signature

Dr. Al Shimaa Zakaria, MD

Co-signed by:

Heba A Abdelbaky (MD)

Reported and electronically endorsed by

heba ahmed Abdel Baqi Abdallah

Pathology lab director:

Dr. Hazem Abdelazim, MD, IFCAP

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مركز نهضة الهرم
4 شارع علوية أول شارع الهرم بجوار دولي, الجزيرة

مستشفى طبية الشيخ زايد
4. الحي الأول - الشيخ زايد أمام كلية الهندسة

MRN: 10126284858
Patient Name: aya ALAA ABDELRAshed MOHAMMED
IC No.: 29402151400728
Location: External Clinic

D.O.B: 15/02/1994
Sex: Female

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id 2609080017
Accession No IMM-26-3860
Sample Type Paraffin Block

Collection Date 08/09/2026 08:48 AM
Received Date 08/09/2026 08:48 AM
Doctor Mohamed Mostafa Mohamed Mostafa

Immunohistochemical Report

pathology number
3253-26

Clinical Data
Rt. breast cancer.

Gross Picture :
Referred block submitted for ER/PR immunostaining.

System Used :
Ventana Benchmark ULTRA autostainer.

Antibody:
Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :
According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:
- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

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مركز رعاية الهرم
4 شارع علوية أول شارع الهرم بحول دادي, الحجرة
مستشفى بحية الشيخ زايد
٤. الحى الأول - الشيخ زايد أمام كلية الهندسة

- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:
Right

Diagnosis :

ER: Negative [0/8].

PR: Negative [0/8].

Signature

Heba A Abdelbaky (MD)

Reported and electronically endorsed by

heba ahmed Abdel Baqi Abdallah

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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MRN: 10126284858
 Patient Name: aya ALAA ABDELRAHED MOHAMMED
 IC No.: 29402151400728
 Location: External Clinic

D.O.B: 15/02/1994
 Sex: Female

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 2609080018
 Accession No IMM-26-3861
 Sample Type Paraffin Block

Collection Date 08/09/2026 08:48 AM
 Received Date 08/09/2026 08:48 AM
 Doctor Mohamed Mostafa Mohamed Mostafa

Immunohistochemical Report

pathology number
 3253-26

Clinical Data
 Rt. breast cancer.

Gross Picture :
 Referred block submitted for HER 2 immunostaining.

System Used :
 Ventana Benchmark ULTRA autostainer.

Antibody:
 Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :
 Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining that is observed in $>10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in $>10\%$ of tumor cells.

(Laterality) breast Cancer:
 Right

Laterality :
 Right

Her2 Result
 Positive [score 3+]

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 4 شارع طلوعة أول شارع الهرم بجوار داري، الجزيرة
 مستشفى مدينة الشيخ زايد
 ٤٠ الحي الأول - الشيخ زايد أمام كلية الهندسة

Signature

Heba A Abdelbaky (MD)

Reported and electronically endorsed by

heba ahmed Abdel Baqi Abdallah

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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4 شارع طلوية أول شارع الهرم بحوار داري , الجزيرة
مستشفى بنية الشيخ زايد
٤٠ الحى الأول - الشيخ زايد أمام كلية الهندسة

